



Armada Kalamunda Group

LUNTERS (Liverpool University Neuroleptic Side Effect Rating Scale) – Using for Monitoring Neuroleptic Side Effect Guideline

1. Purpose

The Liverpool University Neuroleptic Side Effect Rating Scale (LUNTERS) is a tool used to measure the side effects of neuroleptic medication. LUNTERS should be completed for all patients, prescribed neuroleptic medication, if there is no current evidence of a LUNTERS questionnaire in the health care record.

2. Guidelines

The LUNTERS tool is to be utilised within 28 days of prescribing any neuroleptic medications. Scoring of the completed LUNTERS can be utilised to ascertain severity of symptoms, however scoring is not essential for actions to be taken as a result of the completion of the questionnaire.

The LUNTERS tool can be utilised as prompt for medications not in the neuroleptic medication scope.

If no side effects are noted, or are noted but without distress to the patient, the LUNTERS will be repeated within three months.

- Further neuroleptic medication is prescribed, and the doses significantly increased, or side effects noted within the three-month period, LUNTERS is to be repeated within 28 days of the change. Scores which fall into the medium (41-89), High (81-100) Very High (>101) ranges are to be escalated to the treating doctor. The treating doctor will document a plan within the health care record for management
- Side effects are noted as causing distress to the patient, the LUNTERS should be repeated 3-monthly. Any side effect which is causing distress irrespective of score should be escalated to the treating doctor and reviewed.

Documentation in the health record should include:

- Findings and defined actions.
- Adjustment to the medication profile form and Best Practice prescribing data base as required.
- The addition of an alert sticker on the medication chart/s and medication profile form as required.
- Management plan to reflect findings and actions to manage
- Completion date documented in the metabolic screening assessment form – AKMR169.5
- The responsibility for completion of the LUNTERS is:
- Community Clinic –
 - the treating Doctor at the first appointment post the neuroleptic prescription; for consumer to complete prior or post appointment.

- Care coordinator to follow up completion by consumer and report findings to treating doctor.
- the Care Coordinator within the 3-month period.
- Tracking of the required completion date will be facilitated via the PSOLIS Management Plan.

3. Legislative context

The following documents are required to give effect to this procedure:

[Medicines and Poisons Act 2014](#)

[HDWA State-wide Standardised Clinical Documentation SSCD for Mental Health Services OD 0526/14](#)

[East Metropolitan Health Service \(EMHS\) Consent EMHS: 5](#)

[EMHS Patient Identification and Procedure Matching EMHS: 21](#)

4. Supporting information

[AMHS Assessment and Admission Documentation Requirements](#)

[AMHS Assessment Treatment Team Process Manual](#)

[AMHS Clinical Treatment Team Process Manual](#)

[EMHS Care Coordination in Mental Health Services](#)

5. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated with documentation audits and incident review processes.

6. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 4 – Action 4.2
- Standard 6 – Action 6.5

National Standards for Mental Health Services (2010):

Criterion 2.4 The MHS minimises the occurrence of adverse medication events within all MHS settings.

Chief Psychiatrist's Standards for Clinical Care (2015):

- Care Planning
- Physical Health Care of Mental Health Consumers

7. References

1. Australian and New Zealand journal of MH nursing 2000 9, 166, 176.
2. Gray, R., Wykes, T., Parr, A.M., Hails, E. & Gournay, K. (2001). The use of outcome measures to evaluate the efficacy and tolerability of antipsychotic medications: a comparison of Thorn graduate and CPN practise. *Journal of Psychiatric and Mental Health Nursing*.

3. Day, J.C., Wood, G., Dewey, M., Bental, R.P., (1995) A self rating scale for measuring neuroleptic side-effects: validation in a group of schizophrenic patients. *British journal of Psychiatry* 166,650-653.
4. DoH 1999 The National Service Framework for Mental Health – Department of Health. London.
5. Morrison, P., Meehan, T., Gaskill, D., Lunney, P., & Collings, P., (2000) Enhancing case managers skills in the assessment and management of antipsychotic medication side-effects. *Australian and New Zealand Journal of psychiatry* 34,814- 821.
6. Morrison, P., Gaskill, D., Meehan, T., Lunney, P., Lawrence, G. & Collings, P. (2000). The use of Liverpool University Neuroleptic Side-Effect Rating Scale (LUNTERS) in clinical practice. *Australian and NZ Journal of Mental Health Nursing*, 9 (4), 166-176.

8. Glossary

The following definitions are relevant to this policy.

Term	Definition
LUNTERS	Liverpool University Neuroleptic Side Effect Rating Scale

9. Document owner

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10. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V5.0	13/11/2024	13/11/2027	Link added to align with the EMHS Care Coordination in Mental Health Services

11. Authorisation

This guideline has been authorised and issued by the Service Director Mental Health.

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	4/11/2024
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Appendix 1: The Liverpool University Side Effect Rating Scale (LUNSERS)

The Liverpool University Neuroleptic Side Effect Rating Scale (LUNSERS)¹

Questionnaire

The following scale is intended to be self-administered. Please indicate how much you have experienced each of the following symptoms in the last month by ticking a box for each of the 51 items.

Name: _____ Date: _____

	Not at all (0)	Very little (1)	A little (2)	Quite a lot (3)	Very much (4)
1. Rash					
2. Difficulty staying awake during the day					
3. Runny nose					
4. Increased dreaming					
5. Headaches					
6. Dry mouth					
7. Swollen or tender chest					
8. Chilblains					
9. Difficulty in concentrating					
10. Constipation					
11. Hair-loss					
12. Urine darker than usual					
13. Period problems					
14. Tension					
15. Dizziness					
16. Feeling sick					
17. Increased sex drive					
18. Tiredness					
19. Muscle stiffness					
20. Palpitations					
21. Difficulty in remembering things					
22. Losing weight					
23. Lack of emotions					
24. Difficulty in achieving climax					
25. Weak fingernails					

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26. Depression					
27. Increased sweating					
28. Mouth ulcers					
29. Slowing of movements					
30. Greasy skin					
31. Sleeping too much					
32. Difficulty passing water					
33. Flushing of face					
34. Muscle spasms					
35. Sensitivity to sun					
36. Diarrhoea					
37. Over-wet or drooling mouth					
38. Blurred vision					
39. Putting on weight					
40. Restlessness					
41. Difficulty getting to sleep					
42. Neck muscles aching					
43. Shakiness					
44. Pins and needles					
45. Painful joints					
46. Reduced sex drive					
47. New or unusual skin marks					
48. Parts of body moving of their own accord e.g. foot moving up and down					
49. Itchy skin					
50. Periods less frequent					
51. Passing a lot of water					

References:

1. Day JC et al. BJ Psych 1995; 166: 650-653.
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